NCCP 00692 · VERSION 2

Weekly cetuximab + FOLFOX-4

Cetuximab on Day 1 and Day 8. FOLFOX4 starts on Day 1: oxaliplatin, folinic acid and fluorouracil bolus, followed by a 22-hour fluorouracil infusion. Day 2 includes folinic acid, another fluorouracil bolus and a second 22-hour infusion; finish around Day 3. Repeat every 14 days while beneficial and tolerable.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Nerves

Oxaliplatin

Cold-triggered tingling or difficulty with buttons/walking: report.

2 Mouth / swallowing

Fluorouracil

Pain or ulcers affecting eating or drinking need advice.

3 Breathing / reaction

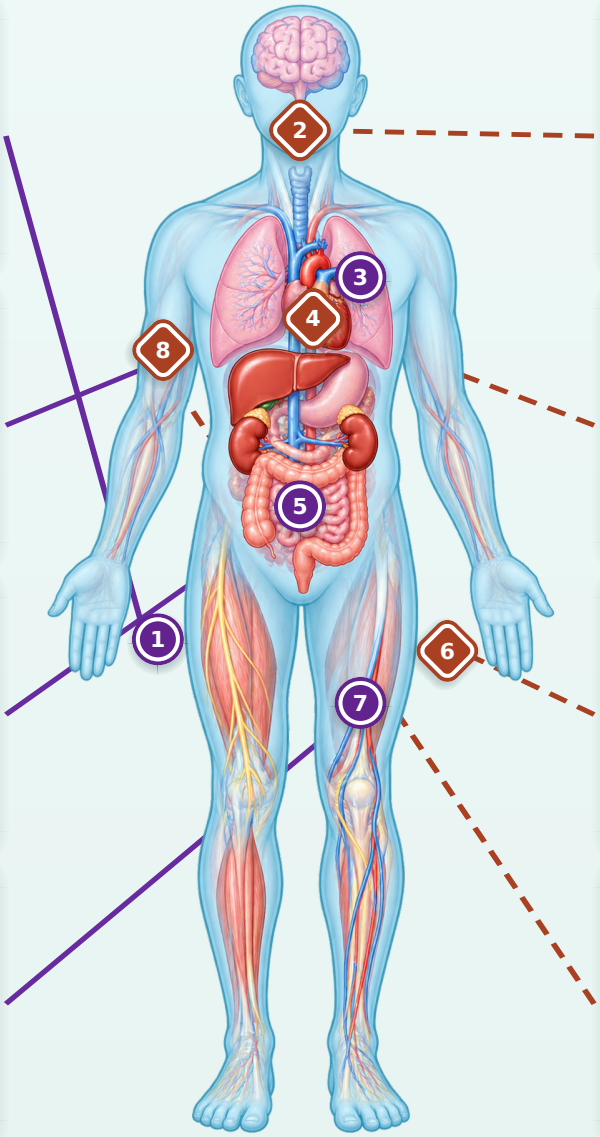
Cetuximab

New breathing difficulty requires prompt assessment.

4 Chest

Fluorouracil

New chest pain needs urgent assessment.



5 Bowel

Treatment

Diarrhoea, increased stoma output or dehydration: call early.

6 Skin / nails

Cetuximab

Report painful rash, cracks or infected skin early.

7 Blood / infection

Treatment

Fever, shivers or sudden illness: contact oncology immediately.

8 Line / pump

Treatment delivery

Pain, swelling, leakage or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Cetuximab

- Rash, dry skin, cracks and sore nails
- Eye inflammation or corneal injury
- Low magnesium, calcium or potassium
- Diarrhoea, dehydration and kidney problems
- Infusion reaction; rare serious lung or skin effects

Oxaliplatin

- Cold-triggered tingling / throat discomfort
- Persistent sensory neuropathy, sometimes lasting after treatment
- Allergic / infusion reaction
- Low blood counts and infection
- Nausea, diarrhoea and fatigue
- Rare serious lung or blood complications

Folinic acid

- Enhances fluorouracil activity and toxicity
- Rare allergic reaction
- Diarrhoea / mouth soreness in combination

Fluorouracil

- Diarrhoea, dehydration and mouth ulcers
- Low blood counts / infection
- Hand-foot skin soreness
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including DPD-related toxicity
- Pump / central-line complications

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Skin, nails and minerals

Start the prescribed skin-care plan early, use moisturiser and sun protection, and report painful or infected skin or nails. Keep mineral blood-test appointments during treatment and for two months after cetuximab.

Bowel symptoms and fluids

Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.

Line and pump plan

Arrange Day 2 infusion renewal and final disconnection around Day 3. This is two separate 22-hour infusions; do not use a 46-hour pump timetable.

Comfort and daily life

Avoid cold drinks and cold exposure as instructed after oxaliplatin; do not suck ice chips during fluorouracil in this regimen. Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

The whole treatment plan

Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Mineral checks continue monthly for two months after the last antibody treatment.

The cetuximab-only visit

Day 8 contains cetuximab only. Its premedicines and observation still matter, but the chemotherapy-day anti-sickness course is not automatically repeated.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00692/

Sources and review status are available online. Follow your own prescription and hospital instructions.